

Venous Thromboembolism

CCG: C-1050 (CCG)

MCG Health
Chronic Care
28th Edition

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Assessment

Q1. Patient: Do you have any blood clots (such as a deep venous thrombosis or pulmonary embolism)?

- Yes [Q2] [Q3] [Q5] [P1]
- No
- Unknown [P2]

Q2. Patient: Has an inferior vena cava (IVC) filter been placed?

- Yes [P1]
- No
- Unknown [P2]

Q3. Patient: Do you take medicine to treat or prevent blood clots? (Check all that apply.)

- Apixaban [Q4] [P1]
- Aspirin [Q4] [P1]
- Clopidogrel [Q4] [P1]
- Dabigatran [Q4] [P1]
- Fondaparinux [Q4] [P1]
- Low-molecular-weight heparin: dalteparin, enoxaparin, or tinzaparin [Q4] [Q5] [P1]
- Rivaroxaban [Q4] [P1]
- Warfarin [Q4] [Q5] [P1]
- None
- Unknown [Q4] [P2]

Q4. Patient: Do you have side effects from the medicine for blood clots? (Check all that apply.)

- Bleeding a lot from cuts, nose, or gums, or bleeding does not stop in 10 to 15 minutes [P1] [P2]
- Blood in stool, urine, vomit, or spit [P1] [P2]
- Bruise easily for no reason [P1] [P2]
- Increased menstrual flow (for patient still having menstrual periods) [P1] [P2]
- Pain or redness at the injection site [P1] [P2]
- Other, specify: _____ [P1] [P2]
- None
- Unknown [P1] [P2]
- Not applicable

Q5. Patient: What doctor checks how you are doing with your blood clot medicine (and checks INR)?

- Doctor's name: _____
- I go to a special clinic (eg, an "anticoagulation clinic").
- I check it and change my doses as needed (self-care).
- INR not checked since: _____ [P1]
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable

Q6. Patient: Do you wear elastic compression stockings?

- Yes
- No [P1]

- Unknown [P2]

Q7. Patient: Do you have any problems with your blood clot treatments? (Check all that apply.)

- Affording medicine or supplies [P1] Financial Status and Benefits  CCG
- Do not have medicine or stockings [P1] Medication Adherence  CCG
- Do not understand how much medicine to take when the doctor changes the dose or timing [P1] Health Literacy  CCG
- Getting to and from appointments [P1] Transportation  CCG
- Need help putting on and taking off stockings [P1] Caregiver Resources Evaluation  CCG
- Remembering to take medicine [P1] Cognitive Impairment  CCG
- Trouble getting injection [P1] Medication Adherence  CCG
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable

Plan of Care

Problems

P1. Blood clot treatment and self-care information needed

- **Goal:** Blood clot treatment and self-care understood
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of signs of new or worsening blood clot problems
 - **EDUCATE:** Activity recommendations, such as(1)(2):
 - Stay as active as possible.
 - Keep legs elevated when sitting or lying down.
 - Try not to cross legs when sitting.
 - Get up and move often. Try not to sit or stand in one place too long.
 - **EDUCATE: When to get emergency help:**
 - Heart attack or stroke signs
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)
 - Bleeding complication (eg, does not stop after 15 minutes, hematemesis, melena)(3)
 - **EDUCATE: When to call the provider(3)(4):**
 - Bleeding problems (eg, unusual bruising or bleeding)
 - Injection site problems (eg, red, swollen)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(5)(6)(7)
 - **COORDINATE:** Follow-up contact to evaluate blood clot care, referral status, and need for further assistance(8)(9)
 - **Goal:** Compression stockings available and worn safely(4)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation for compression stockings, if needed
 - **ASSIST:** Obtaining correct size of compression stockings, if needed(10)
 - **EDUCATE:** Compression stocking use and care(10)(11):
 - Stockings are used to decrease swelling in the legs.
 - Stockings help keep legs from staying swollen.
 - Wear stockings before planned activity such as standing and walking.
 - Wear and remove stockings on a schedule.
 - Skin care important when stockings are off
 - **COORDINATE:** Medical equipment and supplies, if indicated
 - **COORDINATE:** Follow-up to evaluate compression stocking use, referral status, and need for further assistance(8)(9)
 - **SEND:** Wearing compression stockings (Illustrated) (Spanish)
 - **Goal:** Blood clot medication information understood and precautions followed(4)
 - **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling for evaluation of blood clotting medication, if needed
- **EDUCATE:** Clotting prevention medication information:
 - Reason for taking blood clot medication, dosing schedule, and how to manage missed doses
 - Medication problems (eg, side effects, access, administration)
 - Take at same time each day.
 - Medical ID indicating blood clot medication taken(3)
 - Duration of therapy depends on patient's problems.
 - Tell all healthcare providers of use.(3)
 - Tell provider about excessive bleeding signs (eg, bleeding gums, nosebleeds, many bruises).
 - Tell provider about pain in the legs while taking this medication.
 - Get help right away for chest pain or trouble breathing.
 - Medication-related safety
 - Discuss medication with provider if pregnant or plan to become pregnant and taking a blood thinner.[A](3)
- **EDUCATE:** Ways to stay safe while taking blood clot medications(3)(4):
 - Avoid foods that may cause diarrhea or upset stomach, or rapid changes in diet.(12)(13)
 - Floss gently if it does not cause bleeding.
 - Blow nose gently.
 - Use lotion to prevent drying and cracking of skin.
 - Prevent constipation with stool softeners as prescribed.
 - Keep fingernails and toenails short and smooth using nail clipper and file.
 - Use water-soluble lubricants during sexual activity to decrease risk of abrasions.
 - Try not to cause injury (eg, shave with electric razor, use soft-bristled toothbrush, do not use sharp instruments or play high-contact sports).
- **EDUCATE:** Low-molecular-weight heparin self-care, if appropriate(13)(14):
 - May be used during the transition from hospital to outpatient care
 - Heparin is administered by subcutaneous injection.
- **EDUCATE:** Warfarin, if appropriate(4)(14):
 - Dose is monitored using regular blood testing of INR.
 - Warfarin effects (blood thinning) changed by diet. INR can be lowered by eating more than normal amount of foods containing vitamin K.
 - Warfarin interacts with many medications and herbal products.
 - Alcohol may impact INR laboratory values and blood clotting ability. Limit alcohol intake to no more than 1 or 2 drinks a week.
 - Self-monitoring and self-care of blood thinner, if indicated(15)
- **EDUCATE:** When and how to get help for problems related to use of blood-thinning medication(4)
- **COORDINATE:** Laboratory testing, if indicated(3)
- **COORDINATE:** Medical equipment and supplies, if indicated[B]
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Communication between care team members and care plan reconciliation(5)(6)(7)
- **COORDINATE:** Follow-up to evaluate blood clot medication, referral status, and need for further assistance(8)(9)
- **SEND:** Anticoagulants, other than warfarin  (Spanish)
- **SEND:** Warfarin  (Spanish)
- **SEND:** Giving yourself an injection (Illustrated)  (Spanish)

P2. Blood clot information needed

- **Goal:** Blood clot information understood
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation for blood clot risk and need for intervention
 - **EDUCATE:** Get help right away for signs of blood clot.
 - **EDUCATE:** Pulmonary embolism signs(16)(17):
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)
 - Chest pain
 - Heartbeat fast or irregular
 - Fever

- Distended neck veins
- Fainting
- **EDUCATE:** Deep venous thrombosis (DVT) signs(1)(4)(18)(19):
 - Localized tenderness or redness along the area of the affected vein
 - Leg edema, pitting possible
 - Calf swelling 3 cm or larger than asymptomatic side
 - Dilated superficial veins (non-varicose)
- **COORDINATE:** Communication between care team members and care plan reconciliation(5)(6)(7)
- **COORDINATE:** Follow-up contact to evaluate blood clot information and need for further assistance(8)(9)

Evidence Summary

Specialty society: The American College of Chest Physicians and the National Institute for Health and Care Excellence (NICE) do not recommend the use of compression stockings to prevent post-thrombotic syndrome following proximal deep venous thrombosis.(20)(21)

Evidence: A systematic review of 20 randomized controlled trials found that graduated compression stockings are effective in reducing the risk of deep venous thrombosis (DVT) after general and orthopedic surgery, based on high-quality evidence. A reduced risk of proximal DVT is probable with the use of graduated compression stockings, based on moderate-quality evidence; however, there remains a lack of evidence of the effectiveness of graduated compression stockings in medical patients.(22)

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Footnotes

[A] Heparin compounds are the preferred anticoagulants in pregnancy.(3) [A in Context Link 1]

[B] A prothrombin time (INR) home monitoring device may be appropriate.(15) [B in Context Link 1]

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